

SOP-001

Training and Qualification of Study Staff

Standard Operating Procedure | Template | Version 1.2

Site Name:	[Site Name]	Effective Date:	[Enter Date]
Prepared By:	[Enter Name]	Approved By:	[Enter Name]
Review Date:	[Enter Date]	Version:	1.2

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TEMPLATE — Replace all [bracketed placeholders] before use

Principal Investigator Approval

Signature: _____ Print Name: _____ Date: ____/____/____

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1. Objective

The objective of this Standard Operating Procedure (SOP) is to describe the requirements and processes for training, qualifying, and maintaining the competency of all personnel involved in the conduct of clinical research at [Site Name]. This SOP ensures that study staff possess the knowledge, skills, and documented qualifications necessary to protect participant safety, ensure data integrity, and maintain compliance with applicable regulations and Good Clinical Practice (GCP).

2. Scope

This SOP applies to all personnel who participate in any aspect of clinical trial conduct at [Site Name], including but not limited to: Principal Investigators (PIs), Sub-Investigators, Clinical Research Coordinators (CRCs), regulatory staff, data entry personnel, laboratory technicians, and any other individual listed on the Delegation of Authority (DOA) log. This SOP covers initial training, ongoing competency assessment, retraining, and documentation requirements across all active and future studies.

3. Responsibilities

3.1 Principal Investigator (PI)

- Overall responsibility for ensuring all delegated study staff are qualified by education, training, and experience to perform their assigned tasks
- Maintains a current Delegation of Authority (DOA) log for each study
- Reviews and approves all training documentation before staff perform delegated activities
- Conducts or oversees competency assessments and authorizes retraining when deficiencies are identified

3.2 Clinical Research Coordinator (CRC) / Study Staff

- Completes all required training before performing any study-related activities
- Maintains personal training records and provides copies for the regulatory binder
- Notifies the PI or site management when training certifications are expiring or expired
- Participates in ongoing competency assessments and retraining as directed

3.3 Site Management / Quality Assurance

- Establishes and maintains a centralized training tracking system
- Conducts periodic audits of training records for completeness and currency
- Identifies training gaps and coordinates retraining activities
- Ensures training records are audit-ready at all times

3.4 Sponsor / CRO

- Provides protocol-specific training materials and conducts Investigator Meetings
- Provides access to and training on sponsor-required systems (EDC, IVRS/IWRS, eConsent)
- Verifies staff qualifications during monitoring visits

4. Procedures

ICH E6(R3) -- Quality Management System: E6(R3) requires that stakeholder competency be identified as a Critical to Quality (CtQ) factor during trial planning. Documented training is a foundational element of the Quality Management System (QMS). The adequacy of staff training should be evaluated proportionate to the risk of the activities being performed -- higher-risk activities (e.g., informed consent, IP administration, safety reporting) require more intensive training documentation and competency verification.

4.1 GCP Training Requirements

All study staff must complete GCP training before conducting any study-related activities. GCP training provides the foundational understanding of ethical and regulatory requirements for clinical research.

4.1.1 Initial GCP Certification

- Required before any individual is listed on the DOA log or performs study activities
- Must cover the principles of ICH E6, informed consent, investigator responsibilities, and regulatory requirements

4.1.2 Acceptable GCP Training Providers

Provider	Format	Notes
CITI Program	Online, self-paced	Most widely accepted; institutional affiliation may be required
NCI GCP (National Cancer Institute)	Online, self-paced	Free; accepted by most sponsors and IRBs
TransCelerate GCP	Online, self-paced	Industry-developed; increasingly accepted
Sponsor-provided GCP	Varies	Acceptable when sponsor requires their specific program
Institutional GCP programs	Varies	Acceptable if curriculum covers ICH E6 principles

4.1.3 Refresher Frequency

- GCP refresher training is required every 2-3 years per institutional policy, or more frequently if required by the sponsor protocol or IRB
- Staff must complete refresher training before the current certification expires
- If certification lapses, staff must not perform study activities until refresher is completed and documented

4.1.4 Documentation Requirements

- Certificate must include: staff member name, training provider, course title, completion date, and expiration date (if applicable)
- Original or verified copy filed in the regulatory binder (Section E or equivalent)
- Electronic certificates are acceptable when they can be verified through the provider's system

4.2 Protocol-Specific Training

ICH E6(R3) -- Risk-Proportionate Training: Protocol-specific training should be proportionate to the complexity and risk profile of the study. Studies involving vulnerable populations, complex dosing regimens, invasive procedures, or novel investigational products require more comprehensive training with documented competency assessment beyond simple attendance sign-off.

4.2.1 Timing

- Protocol-specific training must be completed BEFORE staff performs any protocol-specific activities
- No exceptions: staff may not perform study procedures, collect data, or interact with participants for a given protocol until training is documented

4.2.2 Training Content

Protocol-specific training must cover, at minimum:

- Study objectives, design, and endpoints
- Eligibility criteria (inclusion/exclusion)
- Study procedures and visit schedule (Schedule of Activities)
- Investigational product handling, storage, preparation, and administration
- Safety reporting requirements (AE/SAE definitions, timelines, reporting procedures)
- Informed consent process specific to the protocol
- Data collection requirements and source documentation expectations
- Protocol-specific deviations and how to avoid them

4.2.3 Documentation

- Training sign-off sheet with: date of training, trainer name and qualifications, topics covered, and signature of each attendee
- Filed in the regulatory binder alongside the protocol
- Investigator Meeting attendance records (if applicable)

4.2.4 Retraining Triggers

Retraining is required when any of the following occur:

- Protocol amendments that change study procedures, eligibility, or safety reporting
- New safety information (e.g., updated Investigator's Brochure, DSMB recommendations, IND Safety Reports requiring procedural changes)
- Protocol deviations attributed to knowledge gaps or misunderstanding of requirements
- Extended absence from study conduct (greater than 6 months)

4.2.5 Mid-Study Staff Additions

When new staff join an active study:

- Complete all training requirements (GCP, protocol-specific, system-specific) before performing any study activities
- Receive training on all current protocol amendments, not just the original protocol
- Review cumulative safety data and any protocol clarification letters or sponsor memos
- Onboarding training documented with the same rigor as initial study training (see Section 4.8)

4.3 Delegation of Authority (DOA) Log Management

The DOA log is a controlled document that serves as the definitive record of which staff members are authorized to perform specific study tasks.

4.3.1 PI Responsibility

- The PI is personally responsible for maintaining a current and accurate DOA log for each study
- The PI must ensure that no individual performs a study task unless they are listed on the DOA log for that task AND have completed the required training

4.3.2 Required Fields

Each DOA log entry must include:

Field	Description
Staff name	Full legal name
Role/Title	e.g., Sub-Investigator, CRC, Regulatory Coordinator
Delegated tasks	Specific tasks authorized (use task codes if sponsor-provided)
Training completion dates	GCP date, protocol training date, system training dates
Start date	Date delegation becomes effective
End date	Date delegation is revoked (blank if active)
Staff signature and date	Acknowledgment of delegated responsibilities
PI signature and date	Authorization of delegation

4.3.3 Update Triggers

The DOA log must be updated when:

- New staff are added to the study team
- Staff depart or are removed from study activities
- Role changes or additional tasks are delegated to existing staff
- Protocol amendments add new procedures requiring delegation
- Staff qualifications change (e.g., license expiration, loss of certification)

4.3.4 Timing of Updates

- The DOA log must be updated BEFORE staff performs any newly delegated tasks
- Retroactive additions to the DOA log are protocol deviations and must be reported per institutional and sponsor requirements

4.3.5 Version Control

- Each update constitutes a new version of the DOA log, signed and dated by the PI
- Superseded versions are retained in the regulatory binder; they are never destroyed or overwritten
- Sponsor-provided DOA log templates take precedence over site templates when available

4.4 System-Specific Training

4.4.1 Electronic Data Capture (EDC)

- Complete sponsor-provided EDC training before performing any data entry
- Training must cover: system navigation, data entry conventions, query resolution, electronic signatures, and audit trail awareness
- Completion certificates filed in the regulatory binder

4.4.2 Interactive Voice/Web Response System (IVRS/IWRS)

- Training must be completed before the first participant randomization or drug dispensing event
- Covers: participant registration, randomization procedures, drug kit assignment, unblinding procedures (emergency only)
- Completion certificates filed in the regulatory binder

4.4.3 eConsent Platform (if applicable)

- Training on electronic consent workflow, participant authentication, version management, and technical troubleshooting
- Staff must be able to demonstrate the consent process before using eConsent with participants

4.4.4 CTMS and Site-Specific Systems

- Training on internal Clinical Trial Management System (CTMS) for visit scheduling, milestone tracking, and regulatory document management
- Training on site-specific electronic medical record (EMR) systems relevant to study conduct
- Training on any site-specific tools for document management, communication, or reporting

4.4.5 Documentation

- All system-specific training completion records (certificates, sign-off sheets, screenshots of completion) filed in the regulatory binder
- Training records must identify the system name, version, date of training, and staff member

4.5 Regulatory and Compliance Training

4.5.1 HIPAA / PHI Handling

- All staff with access to participant protected health information (PHI) must complete HIPAA training
- Annual refresher required
- Training covers: minimum necessary standard, de-identification, breach reporting, and secure handling of PHI in research

4.5.2 Institutional Policies

- Conflicts of Interest (COI) disclosure and management training per institutional requirements
- Research integrity and responsible conduct of research (RCR) training
- Human subjects protection training (may overlap with GCP; institutional requirements may be additive)

4.5.3 Specimen Shipping (if applicable)

- IATA Dangerous Goods Regulations (DGR) training for staff who pack or ship biological specimens
- DOT/IATA training must be refreshed per regulatory requirements (typically every 2-3 years)
- Covers: classification, packaging, labeling, documentation, and emergency procedures

4.5.4 Financial Disclosure

- Training on FDA financial disclosure requirements (21 CFR Part 54)
- Staff must understand what constitutes a disclosable financial interest and the obligation to report changes during the study and for one year after completion

4.6 Training Records Management

ICH E6(R3) -- Stakeholder Competency as CtQ: Under the E6(R3) quality management framework, adequate training of site staff is a Critical to Quality factor. Incomplete or outdated training records represent a quality risk that should be identified, mitigated, and monitored as part of the site's overall QMS. Training record management should be designed for auditability from the outset.

4.6.1 Central Training File

- A central training file is maintained for each study in the regulatory binder (Section E or equivalent, per site filing structure)
- For multi-study staff, a master training file may be maintained centrally with study-specific training documents filed in each study's regulatory binder

4.6.2 Required Documents Per Staff Member

For each individual listed on the DOA log, the following must be on file:

1. Current curriculum vitae (CV) -- signed and dated within the past 2 years
2. Medical license or professional certification (if applicable, with current expiration date)
3. GCP training certificate (current, not expired)
4. Protocol-specific training sign-off sheet
5. System-specific training completion records (EDC, IVRS/IWRS, eConsent, etc.)
6. Financial disclosure form (FDA Form 3454 *certification of no disclosable interests*, or 3455 *disclosure of interests*, or sponsor equivalent)
7. HIPAA training documentation
8. Specimen shipping certification (if applicable)
9. Any additional sponsor-required documentation

4.6.3 Periodic Review Schedule

- Training records are reviewed at least quarterly to verify all certifications and training are current
- Review results are documented, including any deficiencies identified and corrective actions taken
- Quarterly review is timed to allow staff to renew expiring certifications before they lapse

4.6.4 Audit Readiness

- All training records must be organized, complete, and available for monitor, auditor, or inspector review within 24 hours of request
- Electronic records must be backed up and accessible; paper records must be filed in a known, consistent location
- A training records checklist (per staff member, per study) should be maintained and updated at each quarterly review

4.7 Ongoing Competency Assessment

Training attendance alone does not ensure competency. Ongoing assessment verifies that staff can apply their training correctly in practice.

4.7.1 Assessment Methods

Method	Frequency	Applicable To
Direct observation	During onboarding and periodically thereafter	Consent process, IP administration, specimen handling
QC findings review	Ongoing	Data entry accuracy, source documentation completeness
Protocol deviation analysis	Ongoing	All staff -- deviations attributable to specific individuals indicate training gaps
Monitor/CRA feedback	Per monitoring visit	All delegated activities
Self-assessment questionnaires	Annually or per sponsor requirement	Protocol knowledge, GCP principles

4.7.2 Addressing Competency Concerns

When competency concerns are identified through any of the above methods:

1. Document the specific concern (what was observed, when, by whom)
2. Develop a retraining plan tailored to the identified gap
3. Conduct retraining and document completion
4. Perform a competency reassessment (direct observation or knowledge check)
5. PI reviews and signs off that the individual is approved to resume independent activities
6. All documentation filed in the staff member's training record

4.7.3 Annual Performance Review

- Each study staff member's performance is reviewed annually in the context of study conduct quality
- Review considers: protocol deviations attributed to the individual, QC error rates, monitor feedback, and training compliance
- Results inform training plans for the following year

4.8 New Staff Onboarding During Active Studies

Adding staff to an active study requires structured onboarding to ensure participant safety and data consistency are not compromised during the transition.

4.8.1 Minimum Onboarding Checklist

Before a new staff member may perform independent study activities, the following must be completed and documented:

- ☐ GCP certification verified (current, not expired)
- ☐ Protocol-specific training completed (including all amendments to date)
- ☐ System access granted and system-specific training completed (EDC, IVRS/IWRS, etc.)
- ☐ DOA log updated with new staff member and signed by PI
- ☐ CV, license, and financial disclosure collected and filed
- ☐ HIPAA training verified
- ☐ Site-specific SOPs reviewed and acknowledged
- ☐ Shadow period completed (see 4.8.2)
- ☐ Competency assessment completed and signed off by PI or designee

4.8.2 Shadow Period Requirements

- New staff must complete a minimum of 2 supervised study visits before conducting visits independently
- During the shadow period, the new staff member observes and assists but does not serve as the primary coordinator for any visit
- The supervising staff member documents each shadowed visit (date, visit type, activities observed, assessment of readiness)
- PI or senior designee makes the determination that the shadow period is complete and the staff member is ready for independent activities

4.8.3 Temporary Delegation Restrictions

- During the onboarding period, new staff may be listed on the DOA log with restricted delegated tasks
- High-risk activities (informed consent, IP administration, SAE reporting) may require extended supervision at PI discretion
- Restrictions are removed and documented on the DOA log when the PI is satisfied with demonstrated competency

4.8.4 Communication Requirements

- The sponsor and/or CRA must be notified of new staff additions per the protocol and Clinical Trial Agreement requirements
- Updated DOA log, CV, GCP certificate, medical license (if applicable), and financial disclosure must be available for the next monitoring visit
- If required by the sponsor, new staff may need to complete sponsor-specific onboarding or be approved before performing delegated tasks

5. References

- ICH E6(R2) Guidelines for Good Clinical Practice -- Sections 4.1 (Investigator's Qualifications and Agreements), 4.2 (Adequate Resources), 4.9 (Records and Reports)
 - ICH E6(R3) Good Clinical Practice -- Section 4 (Quality Management System: stakeholder competency, CtQ factors, risk-proportionate quality management, documented training as QMS element)
 - 21 CFR 312.53 -- Selecting Investigators and Monitors (investigator qualification and agreement requirements)
 - 21 CFR Part 50 -- Protection of Human Subjects (informed consent requirements)
 - 21 CFR Part 54 -- Financial Disclosure by Clinical Investigators
 - 21 CFR Part 56 -- Institutional Review Boards
 - 45 CFR Part 46 -- Protection of Human Subjects (Common Rule)
 - OHRP Guidance on Investigator Responsibilities -- Informed Consent, Delegation, and Supervision
 - IATA Dangerous Goods Regulations (DGR) -- Training requirements for specimen shipping
 - [Institutional Policy #] -- GCP Training Requirements
 - [Institutional Policy #] -- Conflicts of Interest in Research
 - [Institutional Policy #] -- HIPAA Privacy and Security Training
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6. Revision History

Version	Date	Author	Description of Change
1.0	Initial version		

Approved By: _____

Date: ____/____/____

Title: _____